

Question:

A mental health volunteer is interviewing locals as part of a community outreach program. A 46-year-old man discloses that he has felt sad for as long as he can remember. He feels as though his life is cursed and if something terrible can happen to him, it usually will. He has difficulty making decisions and feels hopeless. He also feels that he has had worsening suicidal ideations, guilt from past problems, energy, and concentration over the past 2 weeks. He is otherwise getting enough sleep and able to hold a job. Which of the following statement best describes this patient's condition?

Answer Choices:

- A. The patient is at risk for double depression.
- B. The patient may have symptoms of mania or psychosis.
- C. The patient is likely to show anhedonia.
- D. The patient likely has paranoid personality disorder.

Reasoning + Answer:

<think>

Finding Reasoning Paths :

- 1 . **Depressed mood + decreased energy + anhedonia + psychomotor retardation** â€” Major Depressive Disorder (MDD).
- 2 . **Depressed mood + hopelessness + suicidal ideation** â€” Major Depressive Disorder (MDD).
- 3 . **History of worsening suicidal ideations + difficulty making decisions** â€” Major Depressive Disorder (MDD).
- 4 . **Decreased ability to concentrate + lethargy** â€” Major Depressive Disorder (MDD).
- 5 . **Psychological symptoms (hopelessness , suicidal ideation) + no psychotic symptoms or manic features** â€” Rule out psychotic disorders (e.g ., schizophrenia , delusional disorder) and rule in depressive disorders .

Reasoning Process :

- 1 . The patient presents with a long-standing history of sadness , difficulty making decisions , and feelings of hopelessness . These are hallmark features of **Major Depressive Disorder (MDD)** .
- 2 . Over the past 2 weeks , the patient has developed worsening suicidal ideations , guilt from past problems , and no change in energy levels . This pattern of worsening symptoms aligns with the acute onset of depressive features , which is consistent with **MDD** .
- 3 . The patient denies psychotic symptoms (e.g ., delusions , hallucinations), mania , or hypomania , making psychotic disorders (e.g ., schizophrenia , delusional disorder) unlikely .
- 4 . The patient's symptoms are not consistent with chronic illnesses like dementia or Huntington's disease , as there is no mention of cognitive decline , motor symptoms , or specific genetic or structural abnormalities .
- 5 . The patient's symptoms are best explained by **Major Depressive Disorder** , given the presence of depressed mood , anhedonia , psychomotor retardation (difficulty making decisions), and suicidal ideation .

Conclusion :

The patient is most likely experiencing symptoms of **Major Depressive Disorder (MDD)** . There is no evidence to suggest psychotic disorders or other conditions such as dementia or Huntington's disease . The provided answer incorrectly attributes symptoms to other conditions , such as Parkinson's disease , which is not supported by the clinical presentation .

</think>

<answer>

The patient is at risk for double depression .

</answer>